



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : امل حسين عبدالحميد على
المحافظة : القاهرة
الحى / المدينة : باب الشعرية
العنوان : ممممممممممممممممممممم
رقم البطاقة : 26507230101405 رقم التليفون : 01149295990
العمر (تاريخ الميلاد) : 23-07-1965 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 27-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: امل حسين عبدالحميد علي National ID: 26507230101405	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
مطالبة على الشخصية



أهل
حسن عبد الحميد علي
٥٥٧
باب الشعبة - القاهرة

٢٦٥٠٧٢٣٠١٠١٤٠٥

JM2487631



٢٠٢٢/٠٨ ٢٦٥٠٧٢٣٠١٠١٤٠٥
ربة منزل
التي
مسجلة
أرملة

البطاقة سارية حتى ٢٠٢٩/٠٨/٠٩





وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
هرمل، سابقا

Name :

Age :

No.:

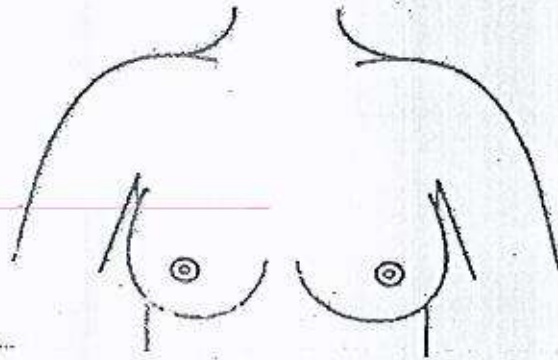
MDT FORM

Clinical data :

Post mast
Left Breast

IX III

CT4, N1, Mo



ER -, PR + 3/2, Her 2/3

path 4AC, 12 Tumor, 3 lymph

LKMM on 20-4-2023

Reason of discussion:

for Tumor
Hercept

MDT Outcome and Plan

Signature :



Pathology Report

Name: *امل حسين عبدالحميد على* Age: 55years.

Clinical data: left breast cancer, received chemotherapy.

Specimen type: left MRM

Date (Received): 20/4/2021

Date(reported): 25/4/2021

Lab. Number:868 /21

Hospital Number: *اقتصادى*

Gross Description:

Left MRM specimen measured 30x15x2cm, with covering skin 20x9cm. Nipple areola unremarkable grossly . cut section revealed multiple scattered rubbery gray white areas admixed with fat, dissection of the axillary fat revealed 12 L.N up to 1x1cm.

Microscopic Examination:

Sections examined revealed totally burnt out tumor tissue, some dilated ducts, adenosis, the stroma was widely fibrosed, and hyalinosed. Nipple and deep margin were free. All submitted lymph nodes were free of tumour deposits, (0/13).

Diagnosis:

Left breast cancer, received chemotherapy, Lt MRM:

- No remnant of viable tumor tissue.
- Complete pathological response (CPR).
- Free Nipple.
- Free deep margin.
- L.N status, 0/13.
- Stage ypT0 ypN0.

Prof. Dr. Gamal Abd El Aty.
Dr. Rana Fathy



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m5329

Female	النوع	امل حسين عبدالحميد علم	المريض
القاهرة	العنوان	55 Y	سن
05/10/2020 10:57	تاريخ استلام العينة		يخ اخذ العينة
11/10/2020 12:42	تاريخ التقرير	مركز اورام دار السلام	لج التحليل
		مركز اورام دار السلام	اية

Primary Sample:

Paraffin Block

Test:

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu: CAP scoring system)

Comment:

Technique:

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by: -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Sample recieved was treated by: -CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result:

ER: NO nuclear staining of tumour cells (core biopsy)

PR: NO nuclear staining of tumour cells (core biopsy)

HER2/neu protein over expression: strong continuous membrane staining of > 10 % of tumor cells (core biopsy).

Ki67: positive nuclear staining of about 20 % of tumour cells (core biopsy)

Referral Lab:

Comment:

Conclusion:

ER: NEGATIVE

PR: NEGATIVE

HER2/NEU PROTEIN OVER EXPRESSION: POSITIVE (SCORE 3+)

#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 20 %

Pathologist:

د/تريزا ماهر

Lab Director:

د/تريزا ماهر

[Signature]

15 El-shaikh Rehan Street, Abeerdeen, Cairo
Tel: 15547948544 - 7950006 Fax: (202)7948544
Mobile: 01144207202
website: <http://www.healpathology.net>
Email: healpathology@healpathology.com
Facebook: <https://www.facebook.com/healpathology>

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بالعينة لمدة 30 يوم من تاريخ استلامها



Patient name: Amal Hussein Abd El-Hamid

Date: 3/10/2021

RIGHT BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense, which may obscure small lesions (ACR C)
- No spiculated masses or suspicious micro calcifications.
- Complementary ultrasound showed no right solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts of the right breast.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions.
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Essential normal study of the right breast (BIRADS 2)

Yours truly

Dr Engy Abdullah

PATIENT NAME: AMAL HUSEN ABD ELHAMID

PATIENT ID #: 2020130256

13

GENDER: Female

ORDER #: 806637

AGE: 55 y

DATE OF SERVICE: 23-Sep-2020 03:45 PM

Report:

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed

23-Sep-2020 05:01 PM

PATIENT NAME: AMAL HUSEN ABD ELHAMID ALI	PATIENT ID #: 2020130256
GENDER: Female	ORDER #: 806635,806636
AGE: 55 y	DATE OF SERVICE: 19-Sep-2020 01:20 PM

Report: - all studies

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS**

1-CHEST

- Multiple enhancing soft tissue lesions are seen in the left breast parenchyma, the largest is seen retroareolar measuring about 2.6 x 2.3 cm in its maximum axial dimensions, along with parenchymatous edematous changes and skin thickening.
- Multiple pathologically enlarged ipsilateral axillary lymph nodes in level I and II, with the largest measuring about 16 x 16 cm in its maximum axial dimensions.
- Apart of a tiny subpleural nodule seen in the apical segment of the right lower lung lobe (likely non-specific), clear rest of both lung fields. No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- A tiny subcentimetric right supraclavicular lymph node.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Evidence of bilateral elastofibroma dorsi.
- Normal CT appearance of the thoracic aorta and the great vessels.

2- ABDOMEN AND PELVIS

- The liver is enlarged in size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions

- seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- A small well-defined cystic lesion is seen intimately related to the greater curvature of the stomach with air loculi seen within measuring 3.2x3.1cm in maximum axial dimensions ??diverticulum.
- Few small subcentimetric gastro-hepatic lymph nodes are seen, likely reactive.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The examined appendicular and axial skeleton shows generalized osteopenic texture of the examined vertebrae with multiple haemangiomas.

Opinion:

- *Multiple left breast enhancing neoplastic mass lesions with ipsilateral pathologically enlarged axillary lymph nodes, likely metastatic.*
- *Hepatomegaly with no CT evidence of metastatic deposits.*

Reported By,

Dr. Ahmed Abdelmoneim, MSc
Dr. Dalia Hussein, MD

19-Sep-2020 06:22 PM

Patient Name: Amal Hussein Abdel Hameed Ali	Date : 08/05/2024
Exam : MAMMOGRAPHY BOTH	Age : 58 Year(s)
Medical ID: 4169/2020	Pat. ID: 28317

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM.

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Signature Dr. Nehal Hamdy

08/05/2024 2:29 PM

Patient Name: Amal Hussein Abdel Hameed Ali	Date : 08/05/2024
Exam : Item 1	Age : 59 Year(s)
Medical ID: MOSTAFA^SHOULDER	Pat. ID: 28317

MRI OF THE RIGHT SHOULDER JOINT

Technique of examination:

- Axial T1 & GR/T2 WIs .
- Coronal T1, T2 & STIR WIs .
- Sagittal T2 WIs .
- No IV contrast was given.

MRI Findings:

- Osteoarthritic changes are seen involving the acromioclavicular articulation with periarticular osteophytic lipping encroaching on the subacromial space.
- The supra spinatus tendon presented focal band of high signal intensity in all pulse sequences reaching to the articular surface with no evidence of complete tendon tear or retraction.
- The subscapularis tendon appears thickened with intrasubstance increased signal , yet no evidence of fibers disruption
- Intact rest of the rotator cuff tendons.
- Osteoarthritic changes are seen involving the glenohumeral articulation with glenoid subchondral cystic changes / ulcerations.
- Pseudocystic change at humeral head is seen.
- Intact glenoid labrum and glenohumeral ligaments.
- Small subdeltoid –subacromion and subcoracoid- bursitis.
- No significant intra articular joint effusion is noted.
- Normal peri-articular musculature with intact intervening fat planes.

conclusion:

- osteoarthritic changes at the acromioclavicular and glenohumeral joints.
- supraspinatus tendon partial thickness tear.
- subscapularis tendoninopathy .
- small subdeltoid –subacromion and subcoracoid-bursitis.

Dr.Hala Sayed, MSc

Signature Prof. Menna Tallah Hanafy

13/05/2024 08:07 م

Patient's Name: Amal Hussein Abdulhamid Ali
Date: 2026-08-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Post left MRM , routine follow up.

Findings:

Right breast.

- Heterogeneously dense breast . (ACR C)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

- *Right breast (BIRADS 1): normal study.*
- *Clear left mastectomy bed and axilla.*

Recommendation:-

- *For annual follow up.*

Best Regards,

DR. Dina Sayed , M.Sc,CUDBIT

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined

with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Suspected Cancer Risk

0 %
0 %
2 %
50 %

98-100%

Patient's Name: Amal Hussein Abdulhamid Ali
Date: 2026-08-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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Recommendation:-

- **For annual follow up.**

Best Regards,

DR. Dina Sayed , M.Sc,CUDBIT

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with recommendations:

Category & recommendations

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0 %

BIRADS 1: Normal study, routine screening

0 %

BIRADS 2: Benign findings, routine screening

2 %

BIRADS 3: Probably benign, for close follow up.

50 %

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

98-100%

BIRADS 6: Pathologically proven malignant